

KNOWLEDGE NEXUS

An Observatory for the Logic of UAE Health Insurance

CONTENT MAP AND WIKI BASIS

A complete structural inventory of the site: every page, section and point

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Scope and Method

Purpose

This document is the structural record of Knowledge Nexus. It enumerates every page of the site, every section within each page, and every discrete point of content beneath those sections. It is intended to serve two functions: as a table of contents for the work as it currently stands, and as the seed structure for a wiki in which each numbered point becomes an article of its own.

Method

The inventory was derived directly from the site source files rather than from any secondary listing. Static pages were parsed for their heading hierarchy; pages whose content is generated at runtime from an embedded dataset — the DRGs Compendium, the Denial Navigator, and the psychiatric IR-DRG tree — were parsed at the level of that dataset, so that no item is lost to rendering. Nothing in this document is summarised, paraphrased, or inferred: each entry reproduces the label as it appears in the source.

Numbering convention

Numbering runs page.section.point. A fourth level (page.section.topic.point) is used in the DRGs Compendium, where content blocks sit beneath topics; in the psychiatric tree, suffixes A, B and C denote cardinal features, resource drivers and verification notes respectively. Every number in this document is stable and may be cited.

Classification of content blocks

Within the DRGs Compendium each block carries one of five labels, reproduced here: Clinical — disease definition and criteria; Coding — code assignment and sequencing; Adjudication — the payer-side test; Coverage — benefit and eligibility rules; Pearl — a single operative caution.

Standing

This is a personal, educational record. It is not an official publication of any authority, and nothing within it constitutes medical, legal, or reimbursement advice. Referenced code systems and standards remain the property of their custodians.

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Front matter

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Inventory by Page

Sections are the top-level divisions of a page. Topics are the intermediate tier, present only where a page has one. Points are the terminal, citable entries.

Page	Sections	Topics	Points
1. Cinematic Opening	0	—	0
2. The Pyramids	1	—	3
3. The Observatory	4	—	10
4. Introduction	4	—	7
5. DRGs Compendium	15	48	138
6. Denial Codes	15	—	23
7. Denial Navigator	8	—	78
8. Psychiatry — Triangulated Lens	1	—	3
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10. Psychiatry × IR-DRG	10	—	109
11. Setting-of-Care	11	—	29
12. Source Corpus	7	—	23
13. Visual Archive	6	—	7
14. Executive Dossier	9	—	45
TOTAL — 14 pages	105	48	567

Pages 1, 2 and 13 are visual surfaces; their low counts reflect design rather than absence of substance. Pages 5 to 12 carry the scholarly weight of the site: 81 sections and 495 points.

Inventory by Page — detail

The pages follow in site order. Each entry reproduces its source filename beneath the heading.

1. Cinematic Opening

index.html

Visual threshold. No structured text content.

2. The Pyramids

pyramids.html

- 2.1 I came from those who built the pyramids.
- 2.2 A doctrine that is structured outlives him.
- 2.3 The Six Chambers

3. The Observatory

observatory.html

Hub page — module cards.

3.1 Knowledge Nexus

3.2 Not a folder of files. A system of thinking.

- 3.2.1 Provenance over assertion
- 3.2.2 Synthesis over collection
- 3.2.3 Refinement over completion

3.3 The route from clinical fact to operational decision.

3.4 Choose an orbit. Enter a working reference.

- 3.4.1 Flagship DRGs Compendium
- 3.4.2 Psychiatric Knowledge System
- 3.4.3 Codes · Global Atlas
- 3.4.4 Setting of Care
- 3.4.5 Gallery
- 3.4.6 About the Author
- 3.4.7 Source Corpus

4. Introduction

introduction.html

4.1 Introduction to the Knowledge Nexus

4.2 The Three Foundational Pillars

- 4.2.1** Provenance over Assertion
- 4.2.2** Synthesis over Collection
- 4.2.3** Refinement over Completion

4.3 Specialized Functional Modules

- 4.3.1** DRGs Compendium
- 4.3.2** Psychiatric Knowledge System
- 4.3.3** Denial Navigator
- 4.3.4** Setting of Care Briefing

4.4 How It Came to Be

5. DRGs Compendium

drgs-compendium.html

5.1 Abu Dhabi Regulatory Codebook

5.1.1 DOH Governance Hierarchy

5.1.2 DOH Claims & Adjudication Rules V2025

5.1.2.1 [Clinical] Encounter definitions (§4.1)

5.1.2.2 [Coding] Claiming methodologies (§4)

5.1.2.3 [Coding] Adjudication layer (§5)

5.1.2.4 [Adjudication] Pre-anesthesia check-up for an inpatient surgery is NOT separately billable — it is included in the inpatient DRG. Unlisted services: use the closest code with an 'UnlistedCodes' observation tag in the eClaim per Routine Reporting Requirements on Shafafiya.

5.1.3 DOH Coding Manual CS v2021 — Authorised Codesets

5.1.3.1 [Clinical] Standard codes — §3.1

5.1.3.2 [Coding] Key DOH principles

5.1.4 Addendum 05 — Inpatient Mental Health Relative Weights

5.1.4.1 [Clinical] Scope

5.1.4.2 [Coding] Application

5.1.4.3 [Adjudication] SOI anomaly on record: Bipolar DRG 19413 shows SOI 2 relative weight (4.7934) exceeding SOI 3 (4.6878) — atypical progression for a severity scale. Flagged for verification against the live Addendum 05 before adjudicating.

5.1.5 Addendum 06 — LAMA Payment Framework

5.1.5.1 [Clinical] LAMA definition

5.1.5.2 [Coding] Payment framework (Addendum 06)

5.1.5.3 [Adjudication] For claims submitted as full IR-DRG where the discharge record documents LAMA, apply the Addendum 06 calculation and adjust accordingly. The documentation burden rests with the provider — absent a LAMA notation, the standard DRG applies.

5.1.6 DOH Circular 2025-18 — Congenital & Hereditary Coverage

5.1.6.1 [Coverage] The coverage rule

5.1.6.2 [Clinical] Cosmetic vs therapeutic test

5.1.6.3 [Coding] Plan-tier overlay

5.2 DRGs Roadmap

5.2.1 Common DRGs — Abu Dhabi IR-DRG Dashboard

5.2.2 IR-DRG Grouper — Solventum Framework

5.2.2.1 [Clinical] Grouper logic

5.2.2.2 [Coding] Payment formula (§4.4.2)

5.2.2.3 [Adjudication] SOI is driven by CC/MCC assignment among secondary diagnoses — every substantiated reportable secondary diagnosis that qualifies as a CC or MCC raises or sustains the SOI. This is

why secondary diagnosis validation is the highest-yield adjudication activity in a DRG market: one unsupported secondary

5.3 Diagnoses Coding Validation

5.3.1 ICD-10-CM Architecture & Conventions (Section I.A)

5.3.1.1 [Clinical] Code structure and edition cycle

5.3.1.2 [Coding] Key conventions

5.3.2 Section I.B — General Coding Guidelines Reference

5.3.2.1 [Adjudication] I.B.2 (specificity) and I.B.19 (no assuming) are the two highest-yield adjudication leverage points — most challengeable claims fail at least one. A claim surviving both is significantly more defensible.

5.3.3 Integral Signs & Symptoms in Detail (I.B.5)

5.3.3.1 [Clinical] The rule

5.3.4 Uncertain Diagnosis — Setting Rules (I.B.14 / II.H / III.C / IV.H)

5.3.4.1 [Clinical] The ruled-out rule

5.3.5 Post-Procedural Complications (I.B.16 / I.A.19)

5.3.5.1 [Clinical] Two-part documentation requirement

5.3.5.2 [Adjudication] D62 (acute post-haemorrhagic anaemia) requires documented acute blood loss — not inferred from haemoglobin. Apply four guidelines together: I.B.16 (post-op complication documentation), I.A.19 (no assuming from labs), I.B.14 (physician must name it), and UHDDS reportability (it must have affected car

5.3.6 Reportable Diagnosis Validation — UHDDS · MEAT · Defensive Trio

5.3.6.1 [Adjudication] The three frameworks operate in sequence: UHDDS sets the reportability bar → MEAT is the tool used to find documentation clearing that bar → the Defensive Trio is the payer's mechanism for identifying where that bar was not cleared. A claim surviving all three is defensible; a claim failing any one

5.3.7 Clinical vs Coding Validation

5.3.7.1 [Adjudication] A claim can be clinically valid but mis-coded (correct diagnosis, wrong code or sequence). A claim can be correctly coded but clinically unsupported (right code, no clinical basis). These are different failures requiring different remedies and different appeal pathways. Always identify which lens fa

5.4 Procedures & HCPCS

5.4.1 3.1 — CPT-4 Categorization

5.4.1.1 [Clinical] Category I structure — six sections

5.4.2 3.2 — NCCI Edits

5.4.2.1 [Clinical] Three core NCCI principles

5.4.2.2 [Coding] UAE / DOH application

5.4.3 3.3 — HCPCS Level II Overview

5.4.3.1 [Clinical] Definition and purpose

5.4.3.2 [Clinical] C-codes — device and supply pass-through

5.4.4 3.4 — DOH-Adopted HCPCS Modifiers & Edits

5.4.4.1 [Coding] Abu Dhabi Simple Edits (§5.3)

5.4.4.2 [Coding] Hospital-Acquired Conditions (§5.5)

5.4.5 3.5 — Add-On Payment Rules for High-Cost HCPCS

5.4.5.1 [Clinical] Two add-on payment tracks

5.4.5.2 [Coding] Costing only vs Payment + Costing

5.4.5.3 [Coding] How to determine payment eligibility

5.4.5.4 [Adjudication] High-cost HCPCS add-on is one of the highest-value adjudication items in inpatient claims. Claims submitted with high-cost HCPCS as a separate payable line require verification that (a) the code is on the approved list, (b) the item was actually used in the patient's care, and (c) the encounter type

5.4.6 3.6 — LAMA Payment Rule

5.4.6.1 [Coding] Three requirements for LAMA to apply

5.5 Sepsis

5.5.1 Sepsis — clinical definition & adjudication framework

5.5.1.1 [Clinical] Sepsis-3 definition

5.5.1.2 [Clinical] SOFA score — what it measures and how it must be applied

5.5.1.3 [Coverage] Preauthorisation — suspected sepsis

5.5.1.4 [Coverage] Final adjudication — proven sepsis

5.5.1.5 [Clinical] Early sepsis — status in the classification

5.5.1.6 [Adjudication] This card is the opening layer. A deeper semantic layer follows in the next topic — 'Sepsis — scoring tables, sequencing & submission' — with the full SOFA / NEWS-2 / Phoenix component tables, the I.C.1.d sequencing rules and the scoring calculators.

5.5.2 Sepsis — scoring tables, sequencing & submission

5.5.2.1 [Coding] Severity coding

5.5.2.2 [Clinical] Sequencing — ICD-10-CM I.C.1.d

5.5.2.3 [Adjudication] Score SOFA within a single 24-hour window; values attributable to a chronic baseline (CKD creatinine, COPD hypoxia, cirrhotic thrombocytopenia) do not count toward the acute increment. Validate organ-dysfunction linkage before R65.2x and confirm persistent hypotension before R65.21. Sepsis is among

5.5.2.4 [Coverage] Scoring calculators

5.6 Chest

5.6.1 Adult Respiratory Failure

5.6.1.1 [Clinical] Adults — types

5.6.1.2 [Clinical] Adults — timing & compensation

5.6.1.3 [Clinical] Adults — oxygenation severity (P/F · Berlin)

5.6.1.4 [Clinical] Adults — clinical signs

5.6.1.5 [Coding] Coding guidance

5.6.2 Respiratory Distress Syndrome (Surfactant Deficiency)

5.6.2.1 Neonatal RDS · P22.0

5.6.2.2 **[Clinical]** Clinical features

5.6.2.3 **[Coding]** Coding

5.6.2.4 **[Pearl]** Management is CPAP / surfactant / ventilation — distinct from TTN. Mild RDS without escalation may not meet respiratory-failure criteria.

5.6.2.5 Transient Tachypnoea (TTN) · P22.1

5.6.2.6 **[Clinical]** Clinical features

5.6.2.7 **[Coding]** Coding

5.6.2.8 Meconium Aspiration (MAS) · P24.01

5.6.2.9 **[Clinical]** Clinical features

5.6.2.10 **[Coding]** Coding

5.6.2.11 PPHN · P29.3

5.6.2.12 **[Clinical]** Clinical features

5.6.2.13 **[Coding]** Coding

5.6.2.14 **[Pearl]** Management — high FiO₂, ventilation, iNO, inotropes, ECMO if refractory — supports a high-acuity picture.

5.6.2.15 Congenital Pneumonia · P23

5.6.2.16 **[Clinical]** Clinical features

5.6.2.17 **[Coding]** Coding

5.6.3 Ventilation — driven DRG

5.6.3.1 **[Clinical]** What drives the tier

5.6.3.2 **[Coding]** CPT relationships

5.7 Fracture Coding

5.7.1 Fracture sequencing — type to 7-character code

5.7.2 Gustilo–Anderson grades & the 7th character

5.7.2.1 **[Clinical]** Gustilo–Anderson grades

5.7.2.2 **[Coding]** 7th character — phase of care

5.8 Metabolic

5.8.1 Acidosis

5.8.1.1 **[Coding]** ICD-10-CM update (effective 1 October 2025)

5.8.1.2 **[Adjudication]** Summary rule — acidosis is coded only when clinically significant, documented, and requiring evaluation, treatment, or monitoring. It must not be coded when integral to another condition or when it fails UHDDS criteria.

5.8.2 Electrolyte disturbances

5.8.3 Encephalopathy

5.8.3.1 **[Clinical]** Forms

5.8.3.2 **[Coding]** Coding

5.8.4 Acute post-haemorrhagic anaemia

5.8.4.1 [Clinical] Clinical basis

5.8.4.2 [Coding] Guideline references

5.9 Renal

5.9.1 Acute kidney injury (KDIGO)

5.9.1.1 [Clinical] KDIGO 2012 — diagnostic criteria (any one sufficient)

5.9.1.2 [Coding] Coding

5.9.1.3 [Pearl] Anchor AKI to a KDIGO threshold and a documented baseline creatinine. A single elevated value, or a transient rise that corrects rapidly with fluids, may not substantiate AKI as a codeable diagnosis.

5.9.1.4 [Clinical] Creatinine interpretation

5.10 Malignancy

5.10.1 Malignancy — sequencing when admission is for a complication

5.10.1.1 [Coding] Sequencing rule (ICD-10-CM Official Guidelines, Section I.C.2 — Neoplasms)

5.10.1.2 [Coverage] Exception — anaemia

5.11 Hepatic

5.11.1 Ascites in Decompensated Liver Cirrhosis — Secondary Diagnosis

5.11.1.1 [Clinical] Pathophysiological basis

5.11.1.2 [Coding] Coding sequence

5.11.1.3 [Adjudication] The deciding test is what drove the admission. Decompensated cirrhosis is the condition the patient was admitted for — the ascites is its clinical expression. Do not elevate a manifestation to the Principal Diagnosis simply because it is prominent or symptomatic.

5.11.2 Malignant Ascites — Principal Diagnosis

5.11.2.1 [Clinical] Guideline authority — I.C.2 (Neoplasms)

5.11.2.2 [Coding] Coding sequence

5.11.2.3 [Adjudication] The sequencing is the inverse of the cirrhosis scenario. The common principle in both is: code what is actually being treated as the Principal Diagnosis. In cirrhosis the organ disease is being treated and ascites is its manifestation. In malignancy, when the admission is purely for ascites drainage

5.12 Surgeries

5.12.1 Congenital & genetic conditions

5.12.1.1 [Coverage] Coverage rule

5.12.2 Cleft lip & palate

5.12.2.1 [Clinical] Why functional

5.12.3 Septoplasty

5.12.3.1 [Coverage] Medical necessity

5.12.4 Blepharoplasty / ptosis

5.12.4.1 [Clinical] Functional criteria

5.12.4.2 [Coverage] Evidence & covered indications

5.12.5 Strabismus surgery

5.12.5.1 [Coverage] Medical necessity

5.12.6 Varicocelectomy

5.12.6.1 [Coverage] Medical necessity

5.12.7 Epidural — coverage

5.12.7.1 [Coverage] Source rule — obstetric epidural (vaginal delivery)

5.12.7.2 [Coverage] Additional — chronic-pain epidural (separate from the obstetric source rule)

5.13 Specialty Medications

5.13.1 Medication master matrix

5.13.1.1 [Pearl] Daratumumab (anti-CD38) interferes with the indirect Coombs (antiglobulin) test → false-positive antibody screens. Notify the transfusion service so compatibility isn't misread.

5.13.1.2 [Adjudication] Most are SC / IM depot therapies that are administered and the patient released — a day-case admission is rarely justified by route alone.

5.13.2 Key specialty-drug cards

5.13.3 GLP-1 receptor agonists

5.13.3.1 [Clinical] Class & mechanism

5.13.3.2 [Clinical] Recognised indications

5.13.3.3 [Pearl] Predominantly subcutaneous therapies (oral semaglutide excepted); route alone rarely justifies admission.

5.13.4 PCSK9 inhibitors

5.13.4.1 [Clinical] Class & mechanism

5.13.4.2 [Clinical] Recognised indications

5.13.4.3 [Pearl] Subcutaneous dosing intervals range from fortnightly to twice-yearly depending on the agent.

5.14 References

5.14.1 Source corpus

5.14.1.1 [Pearl] DRGs Compendium is a working synthesis built on a strong foundation of accumulated healthcare insurance and clinical experience by Dr. Sherif Alattar. This reference is a distinct tier from the regulatory and grouper authorities below, and underpins the validation logic throughout.

5.14.1.2 [Coding] DOH regulatory framework (Abu Dhabi)

5.14.1.3 [Coding] Coding standards

5.14.1.4 [Coding] Grouper & classification

5.14.1.5 [Coding] Clinical & professional

5.15 Specialty Medications — full agent list (23)

5.15.1 Master matrix — 16 agents

#	Generic · Brand	Class / target	Eligible prescriber
5.15.1.1	Dupilumab · Dupixent	IL-4Rα (IL-4/IL-13)	Dermatology / Pulmonology / Allergy (ENT for CRSwNP)
5.15.1.2	Omalizumab · Xolair	Anti-IgE	Allergy / Pulmonology (Dermatology for urticaria)
5.15.1.3	Mepolizumab · Nucala	Anti-IL-5	Pulmonology / Respiratory (Allergy)
5.15.1.4	Tralokinumab · Adtralza	Anti-IL-13	Dermatology
5.15.1.5	Lebrikizumab · Ebglyss	Anti-IL-13	Dermatology
5.15.1.6	Guselkumab · Tremfya	IL-23 (p19)	Dermatology / Rheumatology
5.15.1.7	Canakinumab · Ilaris	IL-1β	Rheumatology / Immunology
5.15.1.8	Daratumumab · Darzalex	Anti-CD38	Haematology / Oncology
5.15.1.9	Caplacizumab · Cablivi	Anti-vWF	Haematology
5.15.1.10	Crizanlizumab · Adakveo	Anti-P-selectin	Haematology
5.15.1.11	Ribociclib · Kisqali	CDK4/6 inhibitor	Oncology
5.15.1.12	Cerliponase alfa · Brineura	Enzyme replacement (TPP1)	Paediatric Neurology
5.15.1.13	Lanreotide · Somatuline	Somatostatin analogue	Endocrinology / Oncology / GI
5.15.1.14	Octreotide · Sandostatin LAR	Somatostatin analogue	Endocrinology / Oncology / GI
5.15.1.15	Triptorelin · Decapeptyl	GnRH agonist	Oncology / Urology / Endocrinology / Gynaecology
5.15.1.16	Bulevirtide · Hepcludex	NTCP entry inhibitor	Hepatology / Infectious Diseases

5.15.2 GLP-1 receptor agonists — 4 agents

5.15.2.1 Dulaglutide

5.15.2.2 Liraglutide

5.15.2.3 Semaglutide

5.15.2.4 Tirzepatide

5.15.3 PCSK9 inhibitors — 3 agents

5.15.3.1 Evolocumab

5.15.3.2 Alirocumab

5.15.3.3 Inclisiran

6. Denial Codes — as Knowledge Infrastructure

denial-codes.html

6.1 Denial Codes as Healthcare Knowledge Infrastructure

6.2 An anatomy of administrative refusal

6.3 Codes as knowledge infrastructure

6.3.1 Research aims

6.4 Where the vocabulary comes from

6.4.1 From commerce to the clinic: EDI and ASC X12

6.4.2 The semantic layer: CARC, the group codes, and RARC

6.4.3 Gulf adoption and the Shafafiya XML pivot

6.5 A precise taxonomic architecture

6.5.1 Search the adopted denial-code series

6.6 Five layers of the claims stack

6.7 EDI and XML as competing grammars

6.8 The denial-code production cycle

6.9 Source, access, and the confidentiality lifecycle

6.9.1 Public vocabulary versus confidential payload

6.9.2 Intellectual-property discipline

6.10 A sociotechnical framework

6.10.1 Epistemology — the denial code as administrative truth

6.10.2 Bowker & Star — classification as infrastructure

6.10.3 Foucault — the discursive boundary of payable reality

6.10.4 Wittgenstein — the limits of administrative language

6.11 Methodological blueprint

6.12 Colophon of authority

6.13 References & reference architecture

6.14 Five layers of the claims stack

6.14.1 Syntax

6.14.2 Semantic vocabulary

6.14.3 Jurisdictional implementation

6.14.4 Operational adjudication

6.14.5 Transactional payload

6.15 Callout cards

- 6.15.1** Core argument
- 6.15.2** A correction of record — two maintainers, not one
- 6.15.3** Primary source
- 6.15.4** Precise separation
- 6.15.5** The correct confidentiality instruments
- 6.15.6** Philosophical synthesis
- 6.15.7** Inclusion & exclusion boundary

7. Denial Navigator

denial-codes-semantic.html

Offline searchable taxonomy — 78 codes across 8 families.

7.1 Eligibility (ELIG) — 7 codes

- 7.1.1 **ELIG-001** — Patient is not a covered member
- 7.1.2 **ELIG-002** — Services performed after the last date of coverage
- 7.1.3 **ELIG-003** — Services performed prior to the effective date of coverage
- 7.1.4 **ELIG-004** — Services performed by a non-network provider are not covered
- 7.1.5 **ELIG-005** — Lifetime benefit maximum has been reached for this service / benefit category
- 7.1.6 **ELIG-006** — Lifetime benefit maximum has been reached
- 7.1.7 **ELIG-007** — Outpatient MRI claim requires pre-approval; not eligible if pre-approval was not obtained or was denied

7.2 Authorization (AUTH) — 9 codes

- 7.2.1 **AUTH-001** — Authorization number is missing or was not obtained
- 7.2.2 **AUTH-002** — Service is not authorized
- 7.2.3 **AUTH-003** — Prior authorization number is invalid
- 7.2.4 **AUTH-004** — Service(s) performed outside authorization validity date
- 7.2.5 **AUTH-005** — Claim information is inconsistent with pre-certified / authorized services
- 7.2.6 **AUTH-006** — Authorization request overlaps or is within the period of another paid claim / approved authorization
- 7.2.7 **AUTH-007** — Waiting period on pre-existing / specific conditions
- 7.2.8 **AUTH-008** — Inappropriate drug dose — prescription exceeds maximum / minimum dosage per day
- 7.2.9 **AUTH-009** — Prescription out of date (non-controlled: 60-day validity; controlled: 3-day validity)

7.3 Clinical / Coding (CODE) — 15 codes

- 7.3.1 **CODE-001** — Missing, incomplete or invalid principal diagnosis
- 7.3.2 **CODE-002** — Missing, incomplete or invalid service code
- 7.3.3 **CODE-003** — Missing, incomplete or invalid data element
- 7.3.4 **CODE-004** — Diagnosis is inconsistent with the patient's age
- 7.3.5 **CODE-005** — Diagnosis is inconsistent with the patient's gender
- 7.3.6 **CODE-006** — Diagnosis is inconsistent with the procedure
- 7.3.7 **CODE-007** — Diagnosis is inconsistent with the provider type
- 7.3.8 **CODE-008** — Service code is inconsistent with the patient's age
- 7.3.9 **CODE-009** — Service code is inconsistent with the patient's gender
- 7.3.10 **CODE-010** — Activity / diagnosis inconsistent with clinician specialty
- 7.3.11 **CODE-011** — Incorrect DRG calculated
- 7.3.12 **CODE-012** — Encounter type inconsistent with service(s) / diagnosis
- 7.3.13 **CODE-013** — Invalid principal diagnosis (e.g. E-codes used as principal diagnosis)
- 7.3.14 **CODE-014** — Activity / diagnosis is inconsistent with the patient's age / gender
- 7.3.15 **CODE-015** — Activity / diagnosis is inconsistent with the provider type

7.4 Duplicate (DUPL) — 2 codes

7.4.1 DUPL-001 — Duplicate claim — exact same claim submitted more than once

7.4.2 DUPL-002 — Payment already made for same / similar service within set time frame

7.5 Medical Necessity (MNEC) — 6 codes

7.5.1 MNEC-001 — Tests or treatments were not ordered by a Medical Doctor

7.5.2 MNEC-002 — Services performed were not medically necessary

7.5.3 MNEC-003 — Service is not clinically indicated based on good clinical practice

7.5.4 MNEC-004 — Service not clinically indicated based on good clinical practice, without additional supporting diagnoses / activities

7.5.5 MNEC-005 — Service / supply may be appropriate, but is too frequent

7.5.6 MNEC-006 — LASIK not covered — member eligible for optical benefits excluding LASIK

7.6 Non-Coverage (NCOV) — 26 codes

7.6.1 NCOV-001 — Diagnosis(es) is (are) not covered

7.6.2 NCOV-002 — Pre-existing conditions are not covered

7.6.3 NCOV-003 — Service(s) is (are) not covered

7.6.4 NCOV-004 — Elective procedure is not covered

7.6.5 NCOV-005 — Dental treatments are not covered

7.6.6 NCOV-006 — Hearing services are not covered

7.6.7 NCOV-007 — Eye exams and treatments are not covered

7.6.8 NCOV-008 — Behavioral and mental health services, treatments and medications are not covered

7.6.9 NCOV-009 — Gender manipulation services and associated expenses are not covered

7.6.10 NCOV-010 — Experimental or investigational services, treatments, devices and medications are not covered

7.6.11 NCOV-011 — Preventative care is not covered

7.6.12 NCOV-012 — Vaccinations are not covered

7.6.13 NCOV-013 — Immunizations are not covered

7.6.14 NCOV-014 — Allergy services and treatments are not covered

7.6.15 NCOV-015 — Acupuncture, acupressure, hypnotism and other forms of alternative treatment are not covered

7.6.16 NCOV-016 — Ambulance transportation for non-emergency situations is not covered

7.6.17 NCOV-017 — Home Health services are not covered

7.6.18 NCOV-018 — Home Health (Respite) services are not covered

7.6.19 NCOV-019 — Home Health (Private Nurse) services are not covered

7.6.20 NCOV-020 — Services resulting from attempted suicide or self-infliction are not covered

7.6.21 NCOV-021 — Claim overlaps inpatient stay — resubmit only services rendered outside the inpatient stay

7.6.22 NCOV-022 — Service(s) is (are) not performed

7.6.23 NCOV-023 — Service(s) performed outside period of hospitalization / admission

7.6.24 NCOV-024 — Chronic condition service(s) is (are) not covered

7.6.25 NCOV-025 — Service(s) not performed (used after audit)

7.6.26 NCOV-026 — Drug not listed in formulary

7.7 Pricing (PRCE) — 10 codes

- 7.7.1 PRCE-001** — Calculation discrepancy
- 7.7.2 PRCE-002** — Payment is included in the allowance for another service
- 7.7.3 PRCE-003** — Recovery of payment — initial payment made to incorrect provider
- 7.7.4 PRCE-004** — Denial reversed because of medical review
- 7.7.5 PRCE-005** — Service has no contract price
- 7.7.6 PRCE-006** — Consultation within free follow-up period
- 7.7.7 PRCE-007** — Service has no contract price — to be negotiated
- 7.7.8 PRCE-008** — Multiple procedure payment rules incorrectly applied
- 7.7.9 PRCE-009** — Bundled code should be used instead of separate codes
- 7.7.10 PRCE-010** — Co-payment / deductible not collected at point of service

7.8 Timely Filing (TIME) — 3 codes

- 7.8.1 TIME-001** — Timely filing limit exceeded — claim not submitted within contractual timeframe
- 7.8.2 TIME-002** — Requested additional information was not received, or not received within time limit
- 7.8.3 TIME-003** — Appeal procedures not followed or time limits not met

8. Psychiatry — A Triangulated Lens

psychiatry-intro.html

8.1 Introduction to the Psychiatric Diseases Triangulated Lens

8.2 Taxonomy Tree

8.3 Deeper Semantic Layer

9. Psychiatry — Semantic Layer

psychiatry-semantic-layer.html

9.1 Psychiatric Diseases Deeper Semantic Layer

9.2 Licensed IR-DRG logic is proprietary

9.2.1 IR-DRG — Solventum (formerly 3M Health Information Systems)

9.3 These weights come from a published DOH addendum

9.3.1 DOH Addendum 05 to the Claims & Adjudication Rules V2025.1 — Appendix A

9.4 Authorities & sources

9.4.1 ICD-10-CM — CDC / NCHS

9.4.2 CMS MS-DRG Definitions Manual

9.4.3 Solventum IR-DRG

9.4.4 DSM-5-TR — APA

9.4.5 DOH Addendum 05 / Claims V2025.1

9.4.6 Five source PDFs (UP-01–05)

9.5 Three lanes, one validation rule

9.5.1 ICD-10-CM + CMS MS-DRG

9.5.2 IR-DRG DX table / grouper

9.5.3 Addendum 05 · Appendix A

9.6 Abu Dhabi coding frame and exclusions

9.6.1 ICD-10-CM, not

9.6.2 CPT-4 / HCPCS

9.6.3 DOH V2025.1 and addenda

9.6.4 Uncertain diagnosis and integral symptoms

9.7 Clinical terms that need visual handling, not just notes

9.7.1 “Neuroses”

9.7.2 Adjustment disorders

9.7.3 Impulse-control disorders

9.7.4 Gender / sexual / psychosexual

9.8 Definitions beneath the labels

9.8.1 Impulse-control disorders · F63

9.8.2 Pyromania

9.8.3 Kleptomania

9.8.4 Trichotillomania

9.8.5 Intermittent explosive disorder

9.8.6 Pathological gambling

9.8.7 Gender, sexual & psychosexual · parked · reference only

- 9.8.8 Transsexualism
- 9.8.9 Dual-role transvestism
- 9.8.10 Transvestic fetishism
- 9.8.11 Paraphilic disorders
- 9.8.12 Sexual dysfunctions
- 9.8.13 Other sexual disorders
- 9.8.14 Terminology bridges
- 9.8.15 “Neuroses”
- 9.8.16 Substance use

9.9 Three distinctions the labels hide

- 9.9.1 Diagnostic safeguards — apply before any category is used as a DRG-navigation proxy
- 9.9.2 Schizophrenia is not all psychosis
- 9.9.3 Major Depression is episodic and higher-threshold
- 9.9.4 Other Depressive Disorders emphasize chronicity or subthreshold presentation

9.10 Where an elegant summary needs a sharper edge

- 9.10.1 Bipolar separates into mania, hypomania, and bipolar depression
- 9.10.2 “Eating disorders” is an umbrella — the criterion layer is per-disorder
- 9.10.3 Other compressed umbrellas that must be unpacked before adjudication

9.11 Relative weights, ALOS & day-outlier trims

- 9.11.1 Mental Health Inpatient payment rules (Addendum 05 §4.4.2)

9.12 Lexicon entries — 13 coded conditions

#	ICD-10	Condition	Status
9.12.1	F63.1	Pyromania	19412 · verify
9.12.2	F63.2	Kleptomania	19412 · verify
9.12.3	F63.3	Trichotillomania	19412 · verify
9.12.4	F63.81	Intermittent explosive disorder	19412 · verify
9.12.5	F63.0	Pathological gambling	19412 · verify
9.12.6	F64.0	Transsexualism	parked
9.12.7	F64.1	Dual-role transvestism	parked
9.12.8	F65.1	Transvestic fetishism	parked
9.12.9	F65	Paraphilic disorders	parked
9.12.10	F52	Sexual dysfunctions	parked
9.12.11	F66	Other sexual disorders	parked
9.12.12	F40–F48	“Neuroses”	terminology
9.12.13	F10–F19	Substance use	terminology

9.13 Label corrections — 6 entries

- 9.13.1 19410 · F20 Schizophrenia is not all psychosis corrected
- 9.13.2 19411 Major Depression is episodic and higher-threshold corrected
- 9.13.3 19414 · F34.1 Other Depressive Disorders emphasize chronicity or subthreshold presentation corrected
- 9.13.4 19413 · F31 Bipolar separates into mania, hypomania, and bipolar depression expanded
- 9.13.5 19416 · F50 “Eating disorders” is an umbrella — the criterion layer is per-disorder expanded
- 9.13.6 multiple Other compressed umbrellas that must be unpacked before adjudication note

9.14 Relative-weight rows — 30 (10 DRGs × base/CC/MCC)

- 9.14.1 Schizophrenia — RW 1.7420
- 9.14.2 Schizophrenia · w/CC — RW 3.4364
- 9.14.3 Schizophrenia · w/MCC — RW 4.6857
- 9.14.4 Major Depression — RW 1.6353
- 9.14.5 Major Depression · w/CC — RW 3.2259
- 9.14.6 Major Depression · w/MCC — RW 4.6656
- 9.14.7 Personality & Impulse Control — RW 0.9545
- 9.14.8 Personality & Impulse Control · w/CC — RW 1.7243
- 9.14.9 Personality & Impulse Control · w/MCC — RW 4.3943
- 9.14.10 Bipolar Disorders — RW 2.4299
- 9.14.11 Bipolar Disorders · w/CC — RW 4.7934
- 9.14.12 Bipolar Disorders · w/MCC — RW 6.6878
- 9.14.13 Other Depressive Disorders — RW 1.3341
- 9.14.14 Other Depressive Disorders · w/CC — RW 2.6317
- 9.14.15 Other Depressive Disorders · w/MCC — RW 3.9588
- 9.14.16 Neuroses Except Depression — RW 1.0195
- 9.14.17 Neuroses Except Depression · w/CC — RW 1.2475
- 9.14.18 Neuroses Except Depression · w/MCC — RW 2.2512
- 9.14.19 Organic Disturbances & Intellectual Disability — RW 1.0174
- 9.14.20 Organic Disturbances & Intellectual Disability · w/CC — RW 1.8864
- 9.14.21 Organic Disturbances & Intellectual Disability · w/MCC — RW 2.9408
- 9.14.22 Childhood Onset Mental Disorders — RW 1.0678
- 9.14.23 Childhood Onset Mental Disorders · w/CC — RW 2.1065
- 9.14.24 Childhood Onset Mental Disorders · w/MCC — RW 3.0010
- 9.14.25 Compulsive Nutrition Disorders — RW 1.1223
- 9.14.26 Compulsive Nutrition Disorders · w/CC — RW 2.2138
- 9.14.27 Compulsive Nutrition Disorders · w/MCC — RW 3.3680
- 9.14.28 Other Mental Disorders — RW 1.1254
- 9.14.29 Other Mental Disorders · w/CC — RW 2.2200
- 9.14.30 Other Mental Disorders · w/MCC — RW 3.0452

10. Psychiatry across DSM-5-TR & IR-DRG

psychiatry-ir-drg-tree.html

10 psychiatric categories, each an IR-DRG with three severity weights.

#	Category	IR-DRG	RW base / w-CC / w-MCC
10.1	Bipolar Disorders	19413	2.4299,4.7934,6.6878
10.2	Schizophrenia Disorders	19410	1.7420,3.4364,4.6857
10.3	Major Depression	19411	1.6353,3.2259,4.6656
10.4	Feeding & Eating Disorders	19418	1.1223,2.2138,3.3680
10.5	Neuroses Except Depression	19415	1.0195,1.2475,2.2512
10.6	Personality & Impulse Control	19412	0.9545,1.7243,4.3943
10.7	Organic Disturbances & ID	19416	1.0174,1.8864,2.9408
10.8	Childhood Onset Disorders	19417	1.0678,2.1065,3.0010
10.9	Other Depressive Disorders	19414	1.3341,2.6317,3.9588
10.10	Other Mental Disorders	19419	1.1254,2.2200,3.0452

10.1 Bipolar Disorders · IR-DRG 19413

Relative weights (base / w-CC / w-MCC): 2.4299,4.7934,6.6878

10.1.A Cardinal features (6)

- 10.1.A.1 Manic episode: elevated, expansive, or irritable mood with increased energy
- 10.1.A.2 Grandiosity or inflated self-esteem
- 10.1.A.3 Decreased need for sleep
- 10.1.A.4 Pressured speech or racing thoughts
- 10.1.A.5 Distractibility, impulsivity, impaired judgment, or high-risk behavior
- 10.1.A.6 Depressive episodes are common but not required for Bipolar I diagnosis

10.1.B Resource drivers (6)

- 10.1.B.1 High safety risk
- 10.1.B.2 Agitation or behavioral escalation
- 10.1.B.3 Restraints / seclusion when clinically justified
- 10.1.B.4 Constant observation
- 10.1.B.5 Medication titration and stabilization
- 10.1.B.6 Longer stabilization with severe mania, psychosis, suicidality, or comorbidity

10.2 Schizophrenia Disorders · IR-DRG 19410

Relative weights (base / w-CC / w-MCC): 1.7420,3.4364,4.6857

10.2.A Cardinal features (6)

- 10.2.A.1 At least two DSM-5-TR Criterion A symptoms
- 10.2.A.2 At least one: delusions, hallucinations, or disorganized speech
- 10.2.A.3 Symptoms for a significant portion of at least 1 month, or less if treated
- 10.2.A.4 Continuous signs of disturbance for at least 6 months
- 10.2.A.5 Functional decline in work, relationships, or self-care
- 10.2.A.6 Negative symptoms may include avolition, flat affect, alogia, asociality, or anhedonia

10.2.B Resource drivers (5)

- 10.2.B.1 Acute psychosis
- 10.2.B.2 Behavioral risk
- 10.2.B.3 Security / safety needs
- 10.2.B.4 High medication load
- 10.2.B.5 Prolonged stabilization

10.2.C Verification note

10.2.C.1 Schizophrenia is one psychotic disorder, not a synonym for all psychoses. Other psychotic disorders require licensed IR-DRG DX table verification.

10.3 Major Depression · IR-DRG 19411

Relative weights (base / w-CC / w-MCC): 1.6353,3.2259,4.6656

10.3.A Cardinal features (6)

- 10.3.A.1 Depressed mood or anhedonia; at least one required
- 10.3.A.2 Symptoms present for at least 2 weeks
- 10.3.A.3 Sleep disturbance or appetite change
- 10.3.A.4 Psychomotor agitation or retardation
- 10.3.A.5 Fatigue, worthlessness, guilt, or impaired concentration
- 10.3.A.6 Suicidal ideation or recurrent thoughts of death

10.3.B Resource drivers (5)

- 10.3.B.1 Suicide / self-harm risk
- 10.3.B.2 Close observation
- 10.3.B.3 Medication and therapy
- 10.3.B.4 Functional impairment
- 10.3.B.5 Severe episode, psychotic features, catatonia, or comorbidity when present

10.4 Feeding & Eating Disorders · IR-DRG 19418

Relative weights (base / w-CC / w-MCC): 1.1223,2.2138,3.3680

10.4.A Cardinal features (5)

- 10.4.A.1 Persistent disturbance in eating or eating-related behavior

- 10.4.A.2** Restriction, avoidance, binge eating, purging, or compensatory behaviors
- 10.4.A.3** Body image disturbance or fear of weight gain when applicable
- 10.4.A.4** Malnutrition, weight instability, or medical compromise in severe restrictive illness
- 10.4.A.5** Psychosocial or physical-health impairment

10.4.B Resource drivers (6)

- 10.4.B.1** Electrolyte instability including phosphate, potassium, or magnesium abnormalities
- 10.4.B.2** Refeeding syndrome monitoring, especially hypophosphatemia
- 10.4.B.3** Cardiac monitoring for bradycardia, arrhythmia, or QTc prolongation
- 10.4.B.4** Structured nutritional rehabilitation
- 10.4.B.5** Nasogastric feeding when indicated
- 10.4.B.6** Medical-psychiatric stabilization and observation

10.4.C Verification note

- 10.4.C.1** Licensed IR-DRG label preserved as Compulsive Nutrition Disorders; visible title uses DSM-5-TR-facing terminology.

10.5 Neuroses Except Depression · IR-DRG 19415

Relative weights (base / w-CC / w-MCC): 1.0195,1.2475,2.2512

10.5.A Cardinal features (5)

- 10.5.A.1** Severe psychological distress without primary psychosis
- 10.5.A.2** Anxiety, panic, phobic, or trauma/stressor-related symptoms
- 10.5.A.3** Obsessions, compulsions, intrusive thoughts, or ritualized behaviors
- 10.5.A.4** Somatic symptom or dissociative presentations without primary psychosis
- 10.5.A.5** Functional impairment or safety concern requiring inpatient stabilization

10.5.B Resource drivers (5)

- 10.5.B.1** Acute anxiety, panic, or trauma-related destabilization
- 10.5.B.2** OCD rituals interfering with nutrition, medication, sleep, or safety
- 10.5.B.3** Dissociative or somatic symptom presentations requiring diagnostic clarification
- 10.5.B.4** Medication optimization and structured psychotherapy
- 10.5.B.5** Functional impairment affecting discharge readiness

10.5.C Verification note

- 10.5.C.1** Neuroses is a legacy DRG label. Adjustment disorders F43.2x may group here or to 19419 depending on the licensed IR-DRG DX table.

10.6 Personality & Impulse Control · IR-DRG 19412

Relative weights (base / w-CC / w-MCC): 0.9545,1.7243,4.3943

10.6.A Cardinal features (5)

- 10.6.A.1** Enduring maladaptive patterns
- 10.6.A.2** Emotional dysregulation
- 10.6.A.3** Impulsive behaviors
- 10.6.A.4** Self-harm gestures

10.6.A.5 Poor impulse control

10.6.B Resource drivers (5)

10.6.B.1 Acute crisis

10.6.B.2 Dangerous impulsive behaviors

10.6.B.3 Self-harm or aggression risk

10.6.B.4 Safety management

10.6.B.5 Long-term instability with repeated crisis presentations

10.6.C Verification note

10.6.C.1 F63 impulse-control codes are an edge zone; verify operational grouping against the licensed IR-DRG DX table.

10.7 Organic Disturbances & ID · IR-DRG 19416

Relative weights (base / w-CC / w-MCC): 1.0174,1.8864,2.9408

10.7.A Cardinal features (5)

10.7.A.1 Psychiatric symptoms due to brain disease, cognitive impairment, or organic etiology

10.7.A.2 Delirium, dementia, or intellectual disability-related presentation

10.7.A.3 Memory deficits or cognitive decline

10.7.A.4 Agitation, wandering, disinhibition, or unsafe behavior

10.7.A.5 Need to link symptoms to medical or neurocognitive causality where applicable

10.7.B Resource drivers (5)

10.7.B.1 Medical causality

10.7.B.2 Behavioral risk

10.7.B.3 Constant supervision

10.7.B.4 Unsafe discharge environment

10.7.B.5 Complex placement or caregiver limitations

10.8 Childhood Onset Disorders · IR-DRG 19417

Relative weights (base / w-CC / w-MCC): 1.0678,2.1065,3.0010

10.8.A Cardinal features (5)

10.8.A.1 Neurodevelopmental or childhood-onset behavioral disorders

10.8.A.2 ADHD, ASD, ODD, or conduct disorder where admission-relevant

10.8.A.3 Hyperactivity, impulsivity, aggression, or severe behavioral escalation

10.8.A.4 Social, emotional, communication, or developmental impairment

10.8.A.5 CAMHS-specific complexity

10.8.B Resource drivers (5)

10.8.B.1 Risk to self, family, or peers

10.8.B.2 Severe behavioral escalation

10.8.B.3 Family and school-system complexity

10.8.B.4 Safeguarding or placement concerns

10.8.B.5 Need for structured multidisciplinary stabilization

10.9 Other Depressive Disorders • IR-DRG 19414

Relative weights (base / w-CC / w-MCC): 1.3341,2.6317,3.9588

10.9.A Cardinal features (6)

- 10.9.A.1** Persistent depressive disorder / dysthymia: chronic depressed mood
- 10.9.A.2** Duration at least 2 years in adults or 1 year in children/adolescents
- 10.9.A.3** Never without Criteria A/B symptoms for more than 2 months
- 10.9.A.4** At least two: appetite, sleep, energy, self-esteem, concentration, or hopelessness
- 10.9.A.5** Other specified or unspecified depressive disorders causing distress or impairment
- 10.9.A.6** PMDD only when supported by the licensed IR-DRG diagnosis table

10.9.B Resource drivers (5)

- 10.9.B.1** Lower baseline intensity
- 10.9.B.2** Escalates with complexity
- 10.9.B.3** Functional impact
- 10.9.B.4** Longer treatment duration than acute depression
- 10.9.B.5** Superimposed major depressive episodes when present

10.10 Other Mental Disorders • IR-DRG 19419

Relative weights (base / w-CC / w-MCC): 1.1254,2.2200,3.0452

10.10.A Cardinal features (5)

- 10.10.A.1** Residual, transient, stress-related, or unclassified psychiatric states
- 10.10.A.2** Adjustment disorders when supported by the licensed IR-DRG diagnosis table
- 10.10.A.3** Stress reactions and somatic symptom-related presentations
- 10.10.A.4** Psychiatric response to severe illness or life events with safety risk or functional collapse
- 10.10.A.5** Other residual psychiatric conditions supported by the licensed IR-DRG diagnosis table

10.10.B Resource drivers (3)

- 10.10.B.1** Usually lower baseline intensity
- 10.10.B.2** Escalates with crisis, safety risk, or comorbidity
- 10.10.B.3** Shorter stabilization unless complicated by behavioral risk or medical complexity

10.10.C Verification note

- 10.10.C.1** Adjustment disorders F43.2x and gender/sexual/psychosexual codes require identity-safe terminology and licensed IR-DRG DX table confirmation when operationally relevant.

11. Setting-of-Care Principles

setting-of-care.html

11.1 Setting of Care Principles

11.2 Care should rise only to the level the patient requires.

- 11.2.1 Clinical necessity governs the setting
- 11.2.2 Care intensity must be distinguished
- 11.2.3 The treatment is not the setting

11.3 Behavior is shaped by information, incentives, and systems.

- 11.3.1 Insurance must manage information asymmetry
- 11.3.2 Provider behavior is shaped by systems

11.4 Management should be proportionate, developmental, and fair.

- 11.4.1 Inform
- 11.4.2 Compare
- 11.4.3 Engage
- 11.4.4 Correct
- 11.4.5 Monitor
- 11.4.6 Escalate

11.5 Design incentives carefully; interpret signals humbly.

- 11.5.1 Payment must not reward avoidable intensity
- 11.5.2 A signal is an invitation to inquiry—not a verdict

11.6 Difference is not dysfunction.

- 11.6.1 Separate variation from unwarranted variation
- 11.6.2 Peer comparison must be credible and fair

11.7 Appropriateness must remain coherent from request to payment.

- 11.7.1 Request
- 11.7.2 Authorization
- 11.7.3 Treatment
- 11.7.4 Documentation
- 11.7.5 Claim
- 11.7.6 Payment
- 11.7.7 Controls must span the lifecycle
- 11.7.8 Prospective stewardship is superior

11.8 Technology should extend judgment—not replace it.

- 11.8.1 Automation should identify; clinicians should judge
- 11.8.2 The burden of control follows the burden of risk

11.9 Stewardship is legitimate only when safety and fairness are indivisible.

11.9.1 Patient safety is the non-negotiable boundary

11.9.2 Due process is itself an insurance control

11.10 Success is alignment—not denial volume.

11.10.1 Measure behavioral improvement

11.10.2 The ultimate objective is alignment

11.11 Care should follow clinical purpose—not administrative habit or economic architecture.

12. Source Corpus

source-corpus.html

23 authorities across six groups.

12.1 Source Corpus The Authorities Beneath Every Page

12.2 The one boundary that governs the whole corpus

12.2.1 IR-DRG is licensed, proprietary, and not reproduced

12.3 The published rules every claim is adjudicated against

12.3.1 DOH Addendum 05 — Inpatient Mental Health Relative Weights

12.3.2 DOH Coding Manual (CS v2021)

12.3.3 DOH Claims & Adjudication Rules V2025

12.3.4 DOH Addendum 06 — LAMA Framework

12.3.5 DOH Mandatory Tariff

12.3.6 DOH Circular — Congenital / Hereditary & Deformity Coverage

12.4 The classification systems the claims are written in

12.4.1 ICD-10-CM Official Guidelines (FY2026)

12.4.2 CPT-4 / HCPCS Level II

12.4.3 NCCI Edits

12.5 The engine that prices the encounter

12.5.1 Solvendum (3M) IR-DRG — v2.x

12.5.2 CMS MS-DRG Definitions Manual

12.5.3 CDC / NCHS ICD-10-CM

12.6 The benchmarks behind every validation rule

12.6.1 Sepsis-3 Definition

12.6.2 NEWS-2 / qSOFA

12.6.3 KDIGO 2012 Staging

12.6.4 Berlin Criteria

12.6.5 Gustilo–Anderson

12.6.6 West Haven Criteria

12.6.7 ACOG Thresholds

12.6.8 DSM-5-TR (APA)

12.6.9 UHDDS / MEAT

12.7 The judgement layer that binds the sources together

12.7.1 DRGs Analysis / Clinical Validation — Dr. Sherif Alattar

13. Visual Archive (Gallery)

gallery.html

Dashboards and short films — evidence of data-management practice.

13.1 Visual Archive

13.2 Motion

13.2.1 The Psychiatric Landscape

13.2.2 The Dashboard, in Motion

13.2.3 Year in Review 2023

13.3 Static Designs by Sherif

13.3.1 The Department Crest

13.4 Data Visualization

13.4.1 The Savings Dashboard

13.5 Service Heroes

13.5.1 Bupa Arabia — “Service Heroes”

13.6 The Cards

13.6.1 The RDD Process

14. About the Author — Executive Dossier

executive-summary.html

14.1 Dr. Sherif Alattar

14.2 Why this portfolio exists.

14.2.1 Credibility

14.2.2 Transparency

14.3 What This Dossier Contains

14.3.1 Clinical Practice Foundation

14.3.2 AUC Academic Formation

14.3.3 Multipayer Operations

14.3.4 Provider RCM Operations

14.3.5 Cross-Regulatory GCC Practice

14.3.6 Operational & Analytical Architecture

14.4 Career Trajectory

14.5 Skills Developed Across the Milestones

14.5.1 Clinical Governance & Medical-Necessity Assurance

14.5.2 Medical Coding, IR-DRG & Documentation Integrity

14.5.3 Utilization & Medical Management

14.5.4 Care Management, Case Management & Population Health

14.5.5 Clinical Claims Adjudication, Payment Integrity & FWA

14.5.6 Cross-Border Regulatory Compliance & Accreditation (KSA & UAE)

14.5.7 Cross-Sector Payer–Provider–TPA Fluency

14.5.8 Provider Network Engagement & Clinical Behavior Management

14.5.9 Value-Based Care, Healthcare Financing & Medical-Cost Management

14.5.10 Enterprise Risk & Clinical Quality Governance

14.5.11 Performance Intelligence & Healthcare Analytics

14.5.12 Health Data Governance, Knowledge Engineering & Applied AI

14.5.13 Healthcare Operating-Model Architecture & Process Engineering

14.5.14 Design Thinking, Information Design & Executive Communication

14.5.15 Technical Signature

14.6 Leadership Timeline

14.7 Education & Qualifications

14.7.1 MB BCh — Bachelor of Medicine, Bachelor of Surgery

14.7.2 PG Diploma — Healthcare & Hospital Management

14.7.3 PG Diploma — Total Quality Management for Healthcare Reform

14.7.4 Lean Six Sigma Green Belt (LSSGB)

14.7.5 IFCE — Insurance Foundation Certificate Examination

14.7.6 IELTS Academic — Band 7.5

14.8 Recognition

14.8.1 Bupa Arabia — "Service Heroes" Recognition

14.8.2 Total Care Saudi — Appointment as Roving Doctors Department Head

14.9 Document Inventory

14.9.1 Academic Credentials

14.9.2 Employment & Experience

14.9.3 Recognition & Awards

14.9.4 Analytical Architecture

14.9.5 Technical Signature

14.9.6 Savings by Insurance Body

14.9.7 Savings by Provider

14.9.8 Total Savings 2023 — SAR 20M

14.9.9 Geographic Coverage Map

14.9.10 Savings by Doctor

14.9.11 Reports & Roving Visits

14.9.12 Engagement Figures

14.9.13 Classification of Savings

14.9.14 Payer and Provider Healthcare Operations and Optimization Systems.

Colophon

Compiled from the Knowledge Nexus source repository on 30 August 2026. Fourteen pages were parsed: eleven as static markup, three at dataset level. The inventory records 105 sections and 567 points.

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